

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Postnatal Contraception for the treatment of Postnatal Contraception

Summary of NICE / NICE CKS Guidelines for Postnatal Contraception

Overview

- Fertility can return as early as 21 days post-delivery.
- Contraception advice should be provided during postnatal care.
- Progesterone-only methods are safe during breastfeeding.

Assessment

- Delivery date
- Breastfeeding status
- Medical history
- VTE risk
- Previous contraceptive use
- Postnatal complications

Management

- Progesterone-only pill (POP) can start any time postpartum (day 1 or later).
- Depo-Provera typically from 6 weeks postpartum (can be earlier if not breastfeeding).
- UKMEC criteria apply.

Red flags - seek urgent medical advice

- DVT/PE symptoms
- Breast cancer (current)
- Severe liver disease
- Unexplained vaginal bleeding

Patient Group Direction

for the supply/administration of

Desogestrel 75mcg tablets (Cerazette/Cerelle)

for the treatment of

Postnatal Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and
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	requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Postnatal contraception for women aged 16+
Inclusion criteria	Postnatal women aged 16 years and over At least 21 days post-delivery Informed consent obtained UKMEC 1 or 2 criteria met
Exclusion criteria	Known or suspected pregnancy Active thromboembolic disorder Sex hormone-dependent malignancy (current or suspected breast cancer) Severe hepatic impairment or liver tumours Undiagnosed vaginal bleeding Hypersensitivity to desogestrel or any excipients
Cautions	History of breast cancer (more than 5 years ago) Functional ovarian cysts Diabetes Hypertension Migraine

	Depression
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Desogestrel 75 micrograms tablets
Legal category	POM
Storage	Store below 30°C
Route/method of administration	Oral, one tablet daily
Dose and frequency	One tablet daily at the same time each day, continuously (no pill-free break). 12-hour window for missed pills. Start day 21 postpartum (no additional contraception needed) or later (use barrier method for 48 hours).
Quantity to be administered and/or supplied	Up to 3 months' supply (3 x 28 tablets = 84 tablets)
Maximum or minimum treatment period	Continuous until change of contraception or pregnancy
Adverse effects	Common: Irregular bleeding or spotting, headache, mood changes, breast tenderness, nausea, acne, decreased libido, weight gain Uncommon: Alopecia, ovarian cysts, fatigue

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear,

	legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication. Discuss breastfeeding safety and the delayed return of fertility with Depo-Provera.
Follow-up advice to be given to patient or carer	Discuss and explain that irregular bleeding is common with both methods, particularly in the first few months. With Depo-Provera, explain that fertility may take 5-6 months to return after the last injection. Both methods are safe during breastfeeding. Take the tablet at the same time each day (Desogestrel) to maintain effectiveness. Return for repeat injection every 12 weeks (Depo-Provera). Seek medical advice if symptoms worsen or any adverse effects occur. Seek immediate medical attention if symptoms of DVT/PE develop (calf pain, swelling, breathlessness). Report any unexpected vaginal bleeding. Discuss contraceptive options if considering longer-term contraception.

Key references

• NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 • NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources • NICE CKS Guidance: Postnatal Contraception • British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines
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Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25

Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25
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Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Medroxyprogesterone acetate 150mg/ml injection (Depo-Provera)

for the treatment of

Postnatal Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Postnatal injectable contraception for women aged 18+
Inclusion criteria	Postnatal women aged 18 years and over At least 6 weeks post-delivery (or earlier per clinical judgement) Informed consent obtained UKMEC 1 or 2 criteria met
Exclusion criteria	Known or suspected pregnancy Current breast cancer Severe liver disease Undiagnosed vaginal bleeding History of hypersensitivity to medroxyprogesterone acetate or any

	excipients Severe cardiovascular disease
Cautions	Bone mineral density loss with prolonged use (review at 2 years) Weight gain Delayed return of fertility (median 5-6 months after last injection) Breast cancer history Migraine Diabetes Depression Adolescents (bone mineral density concern)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Medroxyprogesterone acetate 150 milligrams per millilitre injection
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Deep intramuscular injection into gluteal or deltoid muscle
Dose and frequency	150 milligrams by deep intramuscular injection Repeat every 12 weeks (± 5 days) First injection from 6 weeks postpartum (or earlier if not breastfeeding and no contraindications)
Quantity to be administered and/or supplied	Single injection per visit
Maximum or minimum treatment period	Every 12 weeks (± 5 days) until change of contraception or pregnancy
Adverse effects	Very common: Menstrual irregularity, weight gain Common: Headache, dizziness, mood changes, acne, abdominal discomfort, decreased libido Long-term: Reduced bone mineral density (usually reversible)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
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Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Key references

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<https://www.nice.org.uk/guidance/mg2>

- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Postnatal Contraception
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

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